



CLINICAL PERFORMANCE LAB
The Cheat Sheet Library

VOL I OF IV

The History Question Framework

Universal questions that score on every iHuman case

History is ~40% of your score. Most students lose points not on what they ask — but on what they forget to ask.

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Vol I of IV

The History Question Framework

Universal questions that score on every iHuman case

Here's the thing about iHuman history scoring most students don't realize:

It's not testing whether you can interview the patient. It's testing whether you can systematically cover every domain that a competent clinician would cover — even when the chief complaint seems narrow.

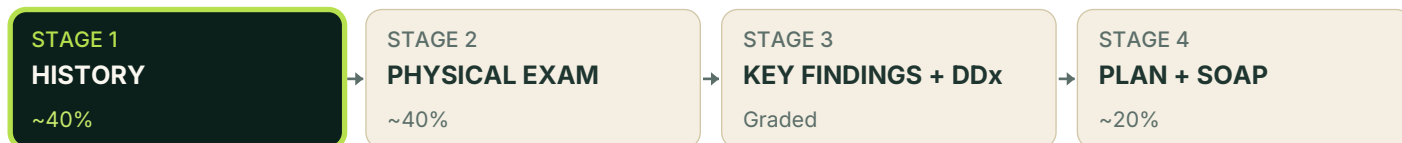
A patient comes in with headaches. You ask about the headaches.

Great. But did you ask about smoking? Family history of cardiovascular disease? Sleep quality? Recent travel? Medications including OTC and herbal? Sexual history if relevant? These are scored on most cases, regardless of the chief complaint. Miss them and you lose 10–20% before you even start.

This cheat sheet is the universal scaffolding that should be in your head before you start any iHuman case.

Use it as a checklist while you're playing. Print it. Pin it. The same framework applies whether your patient is 18 months old with vomiting or 57 years old with hypertension.

THE IHUMAN CASE FLOW · You are here



The Opening Question

Always start with the patient-centered opener. The exact phrasing matters because iHuman scores you on bedside manner.

For an adult patient: "How can I help you today?"

For a pediatric patient (with parent): "How can I help **child's name** today?" — directed to the parent, not the child.

The scored variation: Some cases score a follow-up "What symptom is the most distressing for you?" — known as the pivotal communication question. Missing this is one of the most common point losses across all cases.

OLDCARTS — The HPI Mnemonic

OLDCARTS · The HPI mnemonic

- O** **Onset**
When did it start? Sudden or gradual?
- L** **Location**
Where exactly? Does it move?
- D** **Duration**
How long? Continuous or intermittent?
- C** **Character**
Sharp, dull, throbbing, burning?
- A** **Aggravating/Alleviating**
What makes it better or worse?
- R** **Radiation**
Does it spread anywhere?
- T** **Timing**
Time of day? Frequency?
- S** **Severity**
Rate 1–10. Impact on daily life?

Worked Example: OLDCARTS Applied

Patient presents with chest pain. Here's the OLDCARTS in action — the exact question script:

Onset — "When did the chest pain start? What were you doing when it began?"

Location — "Can you point to where it hurts? Is it in one spot or spread out?"

Duration — "How long does each episode last? Is it constant or does it come and go?"

Character — "How would you describe the pain? Sharp, dull, pressure, burning, crushing?"

Aggravating — "What makes it worse? Exertion, eating, lying down, deep breaths?"

Alleviating — "What makes it better? Rest, position changes, nitroglycerin, antacids?"

Radiation — "Does the pain spread anywhere? Arm, jaw, back, shoulder?"

Timing — "What time of day is it worse? Is there a pattern?"

Severity — "On a scale of 1–10, how bad is it at its worst? How is it affecting your daily life?"

Plus associated symptoms: "Any nausea, sweating, shortness of breath, palpitations, dizziness?"

Plus treatments tried: "Have you taken anything for it? Did it help?"

Past Medical History (PMH) — Required Items

- ✓ Existing medical conditions (chronic and acute)
- ✓ Hospitalizations — when and why
- ✓ Prior surgeries — type, date, complications
- ✓ Allergies — drug, food, environmental (with reaction type)
- ✓ Current medications — Rx, OTC, herbal, supplements
- ✓ Immunizations — current status (especially flu, COVID, pneumococcal, HPV)
- ✓ Screenings — last colonoscopy, mammogram, Pap, DXA scan (age-appropriate)

Family History (FH) — Score-Relevant Conditions

- ✓ Cardiovascular: HTN, CAD, MI, stroke (note age of onset — early = significant)
- ✓ Endocrine: Diabetes Type 1 / Type 2, thyroid disease
- ✓ Cancer: type, age at diagnosis, first-degree relatives
- ✓ Mental health: depression, bipolar, suicide history
- ✓ Neurologic: seizures, dementia, migraines
- ✓ Genetic / hereditary conditions
- ✓ **Premature CVD** (MI before age 55 men, 65 women) — major risk factor; always document

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Social History (SH) — Always-Scored Items

- ✓ **Tobacco** — current, past, never. Pack-years if applicable. (1 PPD × years = pack-years)

- ✓ **Alcohol** — drinks/week, type, pattern
- ✓ **Recreational drugs** — past and current; ask non-judgmentally
- ✓ **Occupation** — current and relevant past exposures
- ✓ **Living situation** — alone, with family, safe environment
- ✓ **Diet** — pattern, restrictions, recent changes
- ✓ **Exercise** — frequency, type, duration
- ✓ **Sleep** — hours, quality, snoring, daytime fatigue
- ✓ **Sexual history** (when relevant) — partners, protection, STI history
- ✓ **Stress** — current stressors, coping, support system

Review of Systems (ROS) — The 14 Domains

iHuman expects you to cover relevant systems even if the chief complaint is focused. Quick screening questions per system:

Constitutional — fever, weight change, fatigue, night sweats

HEENT — headaches, vision, hearing, sinus, throat, dental

Cardiovascular — chest pain, palpitations, edema, dyspnea on exertion

Respiratory — cough, sputum, SOB, wheezing, hemoptysis

GI — appetite, nausea, vomiting, bowel habits, blood in stool, abdominal pain

GU — frequency, urgency, dysuria, hematuria, incontinence

Musculoskeletal — joint pain, swelling, stiffness, weakness

Skin — rashes, lesions, itching, hair/nail changes

Neurologic — headache, dizziness, weakness, numbness, seizures

Psychiatric — mood, anxiety, sleep, suicidal ideation

Endocrine — heat/cold intolerance, polyuria, polydipsia, weight changes

Hematologic/Lymphatic — bruising, bleeding, lymph swelling

Allergic/Immunologic — recurrent infections, allergies

Reproductive — menstrual, pregnancies, contraception, menopause / erectile function

SNOOP4 — Headache Red Flag Screen

SNOOP4 · Headache Red Flag Mnemonic

If ANY are present → DO NOT manage as primary headache. Imaging indicated.

- S** **Systemic signs**
Fever, weight loss, immunocompromise
- N** **Neurologic deficits**
Focal weakness, CN abnormalities, ataxia
- O** **Onset sudden (thunderclap)**
Worst headache of life within seconds
- O** **Onset after age 50**
New headache pattern in older adults
- P** **Papilledema**
Optic disc swelling on fundoscopy
- P** **Postural worsening**
Worse lying down or with Valsalva
- P** **Precipitated by Valsalva**
Cough, sneeze, sex, exertion
- P** **Progressive / Pattern change**
New character, frequency, severity

Lay Language to Clinical Language

iHuman scores you on documentation. Lay language in the EHR loses points. Use clinical terms in your notes even when the patient describes things colloquially.

"Zig-zaggy lights before my headache"	Scintillating scotomas (visual aura)
"Sensitive to light"	Photophobia
"Sensitive to sound"	Phonophobia
"Heart shifted to the side"	Laterally displaced PMI
"Eye changes"	Arteriovenous nicking / hypertensive retinopathy
"Very high blood pressure"	Stage 2 hypertension
"Bad headache with visual changes"	Migraine with aura
"Yellow spots on eyelids"	Xanthelasma

"Grey rings around the eyes"	Corneal arcus
"Trouble breathing at night"	Paroxysmal nocturnal dyspnea (or orthopnea if positional)
"Tired all the time"	Fatigue (note duration and impact on ADLs)
"Bathroom a lot at night"	Nocturia
"Pain spread to the arm"	Pain radiated to the arm
"Throwing up blood"	Hematemesis
"Blood in poop"	Hematochezia (bright red) or melena (dark, tarry)
"Couldn't catch my breath"	Dyspnea / shortness of breath

Common History Traps That Cost Points

⚠ **Skipping the pivotal communication question**

"What symptom is the most distressing for you?" — scored on most cases. Forget this and you lose pivotal-comm points.

⚠ **Forgetting tobacco/alcohol history**

Required on virtually every case, regardless of chief complaint.

⚠ **Not asking about OTC and herbal medications**

Patients self-medicate. The omeprazole, ibuprofen, melatonin, or fish oil they didn't think to mention is often clinically relevant.

⚠ **Missing family history details**

Age of onset matters. "My dad had a heart attack" is different from "My dad had an MI at 48." Premature CVD is a major risk factor and must be documented.

⚠ Asking adult-style questions to pediatric patients

Pediatric history questions go to the parent. The opener is "How can I help **her/him** today?" — not "How can I help you?"

⚠ Stopping at the HPI

OLDCARTS is the start, not the end. You must cover PMH, FH, SH, and ROS even when the case feels focused.

Next steps

The next one in the library covers **Physical Exam — the other 40% of your score**. Many of the same patterns apply: there's a universal set of items that score on nearly every case (cardiac auscultation, lung fields, abdominal exam with bowel sound interpretation, fundoscopy, carotid auscultation, thyroid palpation, PMI). Miss any one of them on a case where it's scored and you lose multiple points.

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